A sustainable service, starting with one partner

Model

Element	Approach
Customer	Private clinics, hospital groups and EMS services first; public facilities through partnerships once POPIA authorisation is in place.
Revenue	A per-facility subscription, plus onboarding and support. Free for patients. Pricing is a hypothesis to validate with the first partner.
Running cost	One hosted instance in Cape Town serves the full product today, under a USD 60 per month budget alarm. Costs grow with facilities, not with patients.
Distribution	Web apps that install like apps, so no app-store dependency; an Android app is the next step for card emulation and offline use.

Position

Alternative	What it does well	Where MediChain differs
Paper folders	Familiar; no infrastructure	Available at the point of care, and every access recorded
Hospital record systems (e.g. OpenMRS, Bahmni)	Established clinical operations	Patient-visible access history and controlled emergency access, built in from the start
Emergency apps (e.g. iER)	Panic alerts and self-reported medical information	Clinician-maintained data behind the emergency summary, read on approved devices, with the read shown to the patient

Comparisons describe focus, not shortcomings: established platforms have their own audit and patient features.

Roadmap

Next	What
Pilot	One partner, one workflow (emergency summary and handover), synthetic training first, then an approved patient pilot.
Regulatory	Information Officer registration, POPIA prior authorisation, legal sign-off.
Product	Android app (phone-to-phone NFC), wider languages, a custom domain, write-once cloud audit sink in production mode.
Team	Today one founder-developer; first hires: a clinical lead and a second engineer, funded through grants (for example NYDA) and pilot revenue.

The ask

A clinic, hospital or EMS partner for a supervised pilot, and introductions to funders who back South African health technology.